4th

UMC Hospitals

Navi Mumbai

DEPARTMENT OF LABORATORY MEDICINE

Name:	**MS. SNEHA RAJU SHINDE**	**UHID:**	UMC-NM-4028	**Collected On:**	03/09/2026 4:17 PM
Age/Sex:	20 Years / Female	**Case No:**	U8620	**Received On:**	03/09/2026 4:17 PM
Consulting Doctor :	**DR. ABHIJIT CHAVAN**			**Reported On:**	03/09/2026 4:22 PM

HEMATOLOGY

TEST	RESULT	UNIT	REFERENCE RANGE
	CBC		
BLOOD COUNT & INDICES			
HEMOGLOBIN	13.5	g/dL	12 - 15
Method : Colorimetric			
TOTAL RBCs	4.55	10^12/L	3.8 - 4.8
Method : Electrical Impedance			
HEMATOCRIT	37.2	%	36 - 46
MCV	**L 76.7**	fL	78 - 100
Method : Calculated			
MCH	27.8	pg	25 - 34
Method : Calculated			
MCHC	**H 36.3**	g/dL	32 - 35
Method : Calculated			
RDW-CV	**H 15.3**	%	11.5 - 14.5
Method : Calculated			
PLATELET COUNT	248	10^9/L	150 - 400
Method : Electrical Impedance			
PCT	0.285	%	-
PDW	15.8		11 - 20
MPV	11.5	fL	7.4 - 12
Method : Calculated			
WBC COUNT	7.09	1E03/mm3	4 - 11
Method : Flow Cytometry By Laser			
DIFFERENTIAL WBC COUNT			
NEUTROPHILS %	60.5	%	40 - 75
Method : Two Angle Scatter			
LYMPHOCYTES %	29.7	%	15 - 45
Method : Two Angle Scatter			

Dr. Nikhil Ningurkar
MD PATHOLOGY
Reg.No. 2012/10/2977

UMC Hospitals

Navi Mumbai

DEPARTMENT OF LABORATORY MEDICINE

Name:	**MS. SNEHA RAJU SHINDE**	**UHID:**	UMC-NM-4028	**Collected On:**	03/09/2026 4:17 PM
Age/Sex:	20 Years / Female	**Case No:**	U8620	**Received On:**	03/09/2026 4:17 PM
Consulting Doctor :	**DR. ABHIJIT CHAVAN**			**Reported On:**	03/09/2026 4:22 PM

HEMATOLOGY

TEST	RESULT	UNIT	REFERENCE RANGE
MONOCYTES %	6.6	%	2 - 10
Method : Two Angle Scatter			
EOSINOPHILS %	2.9	%	0.5 - 7
Method : Two Angle Scatter			
BASOPHILS %	0.3	%	0 - 1
Method : Two Angle Scatter			
NEUTROPHILS #	4.29	10^9/L	2.5 - 8
Method : Calculated			
LYMPHOCYTES #	2.11	10^9/L	1 - 4
Method : Calculated			
MONOCYTES #	0.47	10^9/L	0.2 - 1
Method : Calculated			
EOSINOPHILS #	0.21	10^9/L	0 - 0.5
Method : Calculated			
BASOPHILS #	0.02	10^9/L	0 - 0.1
Method : Calculated			
RBC Morphology	Mild Anisocytosis		
Method : Microscopy			
WBC Morphology	Normal		
Method : Microscopy			
Platelets In Smear	Adequate		-
Method : Microscopy			

Final by - Prachi Valanju

Dr. Nikhil Ningurkar
MD PATHOLOGY
Reg.No. 2012/10/2977

UMC Hospitals

Navi Mumbai

DEPARTMENT OF LABORATORY MEDICINE

Name:	**MS. SNEHA RAJU SHINDE**	**UHID:**	UMC-NM-4028	**Collected On:**	03/09/2026 4:17 PM
Age/Sex:	20 Years / Female	**Case No:**	U8620	**Received On:**	03/09/2026 4:17 PM
Consulting Doctor :	**DR. ABHIJIT CHAVAN**			**Reported On:**	03/09/2026 5:00 PM

BIOCHEMISTRY

TEST	RESULT	UNIT	REFERENCE RANGE
	LFT		
SGPT	**H 1755.1**	U/L	1 - 40
Method : IFCC without pyridoxal phosphate			
SGOT	**H 842.3**	U/L	0 - 32
Method : IFCC without pyridoxal phosphate			
Total BILIRUBIN			
TOTAL BILIRUBIN	**H 8.8**	mg/dl	0.2 - 0.9
DIRECT BILIRUBIN	**H 8.1**	mg/dl	0.1 - 0.3
INDIRECT BILIRUBIN	**H 0.70**	mg/dl	0.1 - 0.4
Method : Calculated			
ALKALINE PHOSPHATASE	**H 158.2**	U/L	35 - 104
TOTAL PROTEIN	8	g/dL	6 - 8
Method : Colorimetric Assay			
SERUM ALBUMIN	**L 3.91**	g/dL	3.97- 4.94 gm/dL
Method : Colorimetric Assay			
Globulin	**H 4.09**	g/dL	1.9 - 3.5
Method : Calculated			
A/G Ratio	**L 0.96**		1.5 - 2.5
Method : Calculated			
GGT	**H 99.1**	U/L	5 - 36
Remark	SGOT & SGPT DONE IN 1:10 DILUTION		

Interpretation :

- Liver Function Test assesses hepatic enzymes, bilirubin, and proteins.
- Alterations may suggest hepatic dysfunction, cholestasis, or metabolic disorders. Interpretation should consider clinical context.

Final by - Prachi Valanju

Dr. Nikhil Ningurkar
MD PATHOLOGY
Reg.No. 2012/10/2977

UMC Hospitals

Navi Mumbai

DEPARTMENT OF LABORATORY MEDICINE

Name:	**MS. SNEHA RAJU SHINDE**	**UHID:**	UMC-NM-4028	**Collected On:**	03/09/2026 4:39 PM
Age/Sex:	20 Years / Female	**Case No:**	U8620	**Received On:**	03/09/2026 4:39 PM
Consulting Doctor :	**DR. ABHIJIT CHAVAN**			**Reported On:**	03/09/2026 4:40 PM

HEMATOLOGY

TEST	RESULT	UNIT	REFERENCE RANGE
	PROTHOMBIN TIME(PT)		
PT			
TEST	**H 18.20**	Secs	9 - 16
CONTROL	13.0	Secs	-
INR	1.4		

Interpretation :

The prothrombin time (PT) and international normalized ratio (INR) are measures of the extrinsic pathway of Coagulation. The INR is used only for patients on stable oral anticoagulant therapy. It makes no significant contribution to the diagnosis or treatment of patients whose PT is prolonged for other reasons.

Increased PT time may be due to :

Factor deficiencies (X, II, V, I), Coumadin (warfarin) Therapy, Liver Diseases (Bile duct obstruction, Cirrhosis, Hepatitis), Hemorrhagic Diseases of the newborn, DIC, Malabsorption, Fibrinolysis, and Vitamin K deficiency.

Interference in PT/INR :

Alcohol, antibiotics, aspirin, cimetidine, thrombin inhibitors (Increase PT) Barbiturates, Oral contraceptives, hormone replacement

therapy (HRT), and Vitamin K (Decrease PT)

Interpretation :

PT is used to evaluate the extrinsic and common pathways of coagulation, which would detect deficiencies of factors II, V, VII, and X, and low fibrinogen concentrations.

INDICATIONS:

- Monitoring vitamin K antagonists (VKA) such as warfarin is the most common indication for PT.
- Evaluation of unexplained bleeding.
- Diagnosing disseminated intravascular coagulation (DIC).
- Obtaining baseline value before initiating anticoagulation therapy.
- Assessment of liver synthesis function and to calculate the model for end-stage liver diseases (MELD) score in liver disease.

REMARKS -

Dr. Nikhil Ningurkar
MD PATHOLOGY
Reg.No. 2012/10/2977

UMC Hospitals

Navi Mumbai

DEPARTMENT OF LABORATORY MEDICINE

Name:	**MS. SNEHA RAJU SHINDE**	**UHID:**	UMC-NM-4028	**Collected On:**	04/09/2026 7:20 AM
Age/Sex:	20 Years / Female	**Case No:**	U8654	**Received On:**	04/09/2026 7:20 AM
Consulting Doctor :	**DR. ABHIJIT CHAVAN**			**Reported On:**	04/09/2026 9:18 AM

HEMATOLOGY

TEST	RESULT	UNIT	REFERENCE RANGE
	PROTHOMBIN TIME(PT)		
PT			
TEST	**H 16.90**	Secs	9 - 16
CONTROL	13.0	Secs	-
INR	1.3		

Interpretation :

The prothrombin time (PT) and international normalized ratio (INR) are measures of the extrinsic pathway of Coagulation. The INR is used only for patients on stable oral anticoagulant therapy. It makes no significant contribution to the diagnosis or treatment of patients whose PT is prolonged for other reasons.

Increased PT time may be due to :

Factor deficiencies (X, II, V, I), Coumadin (warfarin) Therapy, Liver Diseases (Bile duct obstruction, Cirrhosis, Hepatitis), Hemorrhagic Diseases of the newborn, DIC, Malabsorption, Fibrinolysis, and Vitamin K deficiency.

Interference in PT/INR :

Alcohol, antibiotics, aspirin, cimetidine, thrombin inhibitors (Increase PT) Barbiturates, Oral contraceptives, hormone replacement

therapy (HRT), and Vitamin K (Decrease PT)

Interpretation :

PT is used to evaluate the extrinsic and common pathways of coagulation, which would detect deficiencies of factors II, V, VII, and X, and low fibrinogen concentrations.

INDICATIONS:

- Monitoring vitamin K antagonists (VKA) such as warfarin is the most common indication for PT.
- Evaluation of unexplained bleeding.
- Diagnosing disseminated intravascular coagulation (DIC).
- Obtaining baseline value before initiating anticoagulation therapy.
- Assessment of liver synthesis function and to calculate the model for end-stage liver diseases (MELD) score in liver disease.

REMARKS -

Dr. Nikhil Ningurkar
MD PATHOLOGY
Reg.No. 2012/10/2977

UMC Hospitals

Navi Mumbai

DEPARTMENT OF LABORATORY MEDICINE

Name:	**MS. SNEHA RAJU SHINDE**	**UHID:**	UMC-NM-4028	**Collected On:**	04/09/2026 7:20 AM
Age/Sex:	20 Years / Female	**Case No:**	U8654	**Received On:**	04/09/2026 7:20 AM
Consulting Doctor :	**DR. ABHIJIT CHAVAN**			**Reported On:**	04/09/2026 8:57 AM

BIOCHEMISTRY

TEST	RESULT	UNIT	REFERENCE RANGE
	LFT		
SGPT **Method** : IFCC without pyridoxal phosphate	**H 1190.2**	U/L	1 - 40
SGOT **Method** : IFCC without pyridoxal phosphate	**H 460.8**	U/L	0 - 32
Total BILIRUBIN			
TOTAL BILIRUBIN	**H 7.4**	mg/dl	0.2 - 0.9
DIRECT BILIRUBIN	**H 7.2**	mg/dl	0.1 - 0.3
INDIRECT BILIRUBIN **Method** : Calculated	0.20	mg/dl	0.1 - 0.4
ALKALINE PHOSPHATASE	**H 137.5**	U/L	35 - 104
TOTAL PROTEIN **Method** : Colorimetric Assay	6.9	g/dL	6 - 8
SERUM ALBUMIN **Method** : Colorimetric Assay	**L 3.44**	g/dL	3.97- 4.94 gm/dL
Globulin **Method** : Calculated	3.46	g/dL	1.9 - 3.5
A/G Ratio **Method** : Calculated	**L 0.99**		1.5 - 2.5
GGT	**H 85.3**	U/L	5 - 36
Remark	SGPT DONE IN 1:10 DILUTION		

Interpretation :

- Liver Function Test assesses hepatic enzymes, bilirubin, and proteins.
- Alterations may suggest hepatic dysfunction, cholestasis, or metabolic disorders. Interpretation should consider clinical context.

Final by - Prachi Valanju

Dr. Nikhil Ningurkar
MD PATHOLOGY
Reg.No. 2012/10/2977

www.umchospitals.com | +91-22 -41015555 | info.navimumbai@umchospitals.com

UMC Hospitals

Navi Mumbai

DEPARTMENT OF LABORATORY MEDICINE

Name:	MS. SNEHA RAJU SHINDE	**UHID:**	UMC-NM-4028	**Collected On:**	05/09/2026 6:23 AM
Age/Sex:	20 Years / Female	**Case No:**	U8687	**Received On:**	05/09/2026 6:23 AM
Consulting Doctor :	DR. ABHIJIT CHAVAN			**Reported On:**	05/09/2026 7:31 AM

BIOCHEMISTRY

TEST	RESULT	UNIT	REFERENCE RANGE
	LFT		
SGPT	H 841.3	U/L	1 - 40
Method : IFCC without pyridoxal phosphate			
SGOT	H 204.3	U/L	0 - 32
Method : IFCC without pyridoxal phosphate			
Total BILIRUBIN	H 5.6	mg/dl	0.2 - 0.9
TOTAL BILIRUBIN	H 5.4	mg/dl	0.1 - 0.3
DIRECT BILIRUBIN	0.20	mg/dl	0.1 - 0.4
INDIRECT BILIRUBIN			
Method : Calculated			
ALKALINE PHOSPHATASE	H 120.5	U/L	35 - 104
TOTAL PROTEIN	6.2	g/dL	6 - 8
Method : Colorimetric Assay			
SERUM ALBUMIN	L 3.22	g/dL	3.97- 4.94 gm/dL
Method : Colorimetric Assay			
Globulin	2.98	g/dL	1.9 - 3.5
Method : Calculated			1.5 - 2.5
A/G Ratio	L 1.08		
Method : Calculated			
GGT	H 81.4	U/L	5 - 36
Remark	SGPT(ALT) DONE IN 1:10 DILUTION		

Interpretation :

- Liver Function Test assesses hepatic enzymes, bilirubin, and proteins.
- Alterations may suggest hepatic dysfunction, cholestasis, or metabolic disorders. Interpretation should consider clinical context.

Final by - Prachi Valanju

Dr. Nikhil Ningurkar
MD PATHOLOGY
Reg.No. 2012/10/2977

UMC Hospitals

Navi Mumbai

DEPARTMENT OF LABORATORY MEDICINE

Name:	**MS. SNEHA RAJU SHINDE**	**UHID:**	UMC-NM-4028	**Collected On:**	21/09/2026 6:56 AM
Age/Sex:	21 Years / Female	**Case No:**	U9194	**Received On:**	21/09/2026 6:56 AM
Consulting Doctor :	**DR. ABHIJIT CHAVAN**	**Bed No:**	414	**Reported On:**	21/09/2026 7:31 AM
		Ward Name:	GENERAL WARD - 2		

HEMATOLOGY

TEST	RESULT	UNIT	REFERENCE RANGE
	CBC		
BLOOD COUNT & INDICES			
HEMOGLOBIN	**L 10.7**	g/dL	12 - 15
Method : Colorimetric			
TOTAL RBCs	**L 3.69**	10^12/L	3.8 - 4.8
Method : Electrical Impedance			
HEMATOCRIT	**L 28.9**	%	36 - 46
MCV	78.5	fL	78 - 100
Method : Calculated			
MCH	29	pg	25 - 34
Method : Calculated			
MCHC	**H 37**	g/dL	32 - 35
Method : Calculated			
RDW-CV	**H 19.4**	%	11.5 - 14.5
Method : Calculated			
PLATELET COUNT	183	10^9/L	150 - 400
Method : Electrical Impedance			
PCT	0.191	%	-
PDW	16		11 - 20
MPV	10.4	fL	7.4 - 12
Method : Calculated			
WBC COUNT	4.04	1E03/mm3	4 - 11
Method : Flow Cytometry By Laser			
DIFFERENTIAL WBC COUNT			
NEUTROPHILS %	**L 33.4**	%	40 - 75
Method : Two Angle Scatter			
LYMPHOCYTES %	**H 54.2**	%	15 - 45

Dr. Nikhil Ningurkar
MD PATHOLOGY
Reg.No. 2012/10/2977

UMC Hospitals

Navi Mumbai

DEPARTMENT OF LABORATORY MEDICINE

ame:	MS. SNEHA RAJU SHINDE	UHID:	UMC-NM-4028	Collected On:	21/09/2026 6:56 AM
ge/Sex:	21 Years / Female	Case No:	U9194	Received On:	21/09/2026 6:56 AM
onsulting Doctor :	DR. ABHIJIT CHAVAN	Bed No:	414	Reported On:	21/09/2026 7:31 AM
		Ward Name:	GENERAL WARD - 2		

HEMATOLOGY

EST	RESULT	UNIT	REFERENCE RANGE
Method : Two Angle Scatter			
MONOCYTES %	9.4	%	2 - 10
Method : Two Angle Scatter			
EOSINOPHILS %	2.5	%	0.5 - 7
Method : Two Angle Scatter			
BASOPHILS %	0.5	%	0 - 1
Method : Two Angle Scatter			
NEUTROPHILS #	**L 1.35**	10^9/L	2.5 - 8
Method : Calculated			
LYMPHOCYTES #	2.19	10^9/L	1 - 4
Method : Calculated			
MONOCYTES #	0.38	10^9/L	0.2 - 1
Method : Calculated			
EOSINOPHILS #	0.1	10^9/L	0 - 0.5
Method : Calculated			
BASOPHILS #	0.02	10^9/L	0 - 0.1
Method : Calculated			
RBC Morphology	Anisocytosis+		
Method : Microscopy			
WBC Morphology	Lymphocytosis		
Method : Microscopy			
Platelets In Smear	Adequate		-
Method : Microscopy			

Final by - Prachi Valanju

Dr. Nikhil Ningurkar
MD PATHOLOGY
Reg.No. 2012/10/2977